

# SWASTHYA VANI

## Where Health Speaks and We Listen

**TEAM: CASE CRACKERS**

Honey Drolia | Pragya Agarwal | Dishita Jain | Ayushi Prabhakar

## Problem Overview

Rural India faces a severe healthcare crisis despite government initiatives like Ayushman Bharat due to poor infrastructure, lack of providers, mistrust, and systemic failures. Without community-led solutions, the gap will keep growing.

## ROOT CAUSES

70% of India's population lives in rural areas but only 30% of doctors are available there.

### Technological barriers

Poor internet connectivity, Low digital literacy and difficulty in using telemedicine platforms.

### Infrastructure Issues

Poor Roads & Transport. Lack of Local Health Centers, Inadequate clinics and only few doctors willing to work in rural areas.

### Gender Norms

Women may be forbidden from discussing health issues openly, especially related to pregnancy or menstruation.

### Cultural Barriers

Stigma around certain diseases and preference for "jhad-phoonk" or "Totka" over clinical treatment.

## SOLUTION + OPERATIONAL MODEL: HYBRID MODEL

### Campaigns and Training

One month training of local women/youth deliver basic services (first-aid, maternal health, chronic disease follow-up).

### Swasthya Vans

- Free basic health checkup providing services twice a week in every village.
- Medicine inventory control.

### Mobile App (Swasthya Vaani)

- Appointment bookings
- Video and audio consultation with doctors.
- Access to nearby hospitals.
- Emergency alerts.
- Self diagnostic video guides.

### Primary healthcare hubs

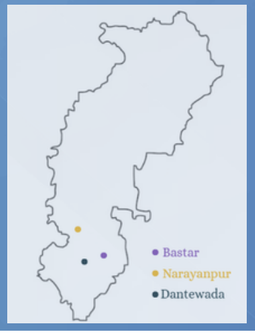
- One in each village.
- Access to be given through Sarpanch.
- Availability of portable diagnostic kits.
- Free basic health checkup.
- Equipment for seasonal diseases.

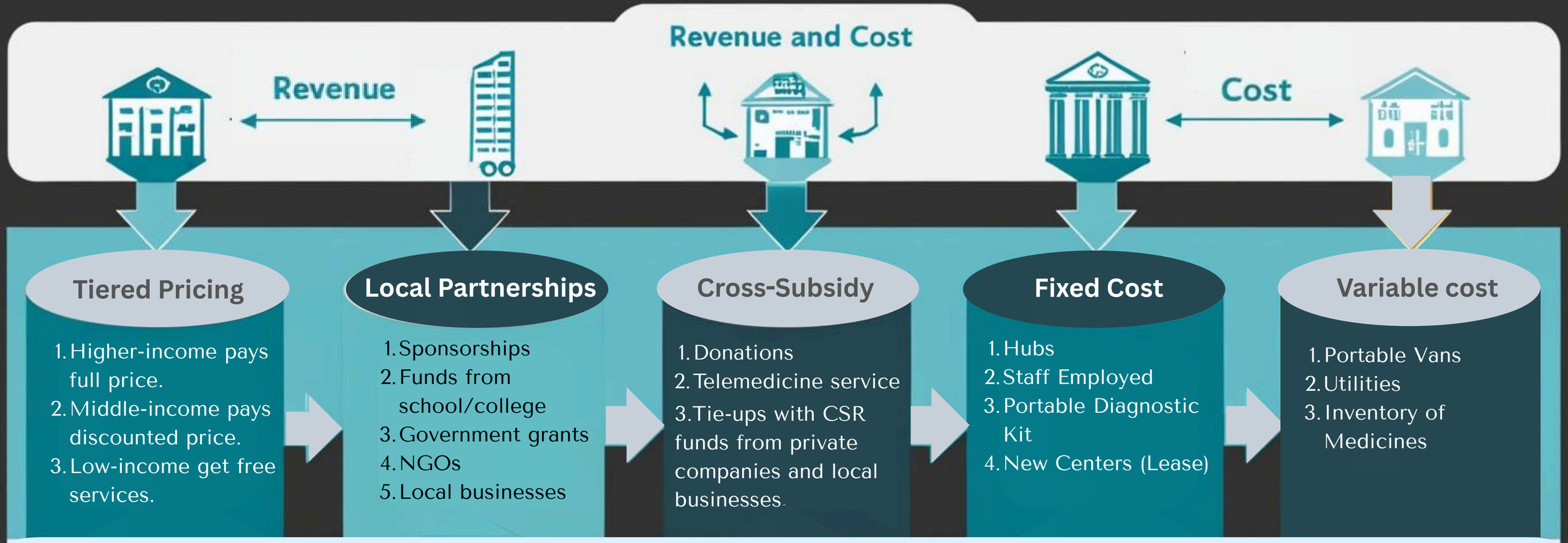
### Role of Sarpanch

- Spreading awareness through Sarpanch as village people often believe local leaders more.
- Giving monetary incentives and recognition to villages and Sarpanch for healthcare excellence.

## TARGET GROUP

Tribal populations and remote rural villages in aspirational districts across India. We will begin implementing our initiatives in some of the most underdeveloped districts of Chhattisgarh, such as: Bastar, Narayanpur and Dantewada. Following the successful establishment from these regions, we plan to expand our efforts to other aspirational districts throughout the country.






## Revenue → Cost → Healthcare delivery → Social Imapct→ Partnerships → (back to Revenue)

The total initial investment required for the venture is estimated at $X million, which will cover infrastructure, equipment, technology, and initial operational costs. The projected revenue for the first year is $Z million, based on the estimated patient volume and pricing model. The annual operational cost in the first year is estimated at $A million, including salaries, medical supplies, and administrative expenses. The break-even point is anticipated to be reached within X years, driven by increasing service demand and optimizing operational efficiencies.

# COST+REVENUE MODEL

# Value Proposition

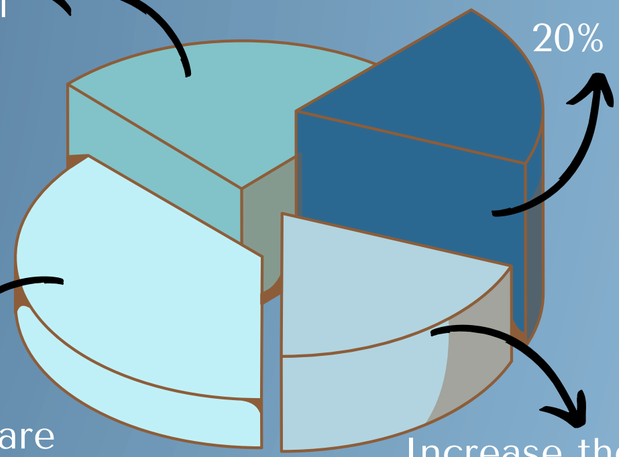
**Built for low-connectivity areas:**
Offline-first tech.

**Gender-sensitive:**
Trained women workers boost maternal health outcomes.

**Affordability:**
Mobile model cuts infrastructure costs.

**Trust and Familiarity:**
Local Saathis bridge cultural gaps.

# Social Impact Metrics



Achieve a patient satisfaction score of 80%+.

Reduce untreated chronic cases by 20% within 2 years.

Increase healthcare access to 60%+ of the population within the first year.

Increase the percentage of people preferring formal healthcare over traditional healers.

# SCALABILITY ROADMAP-SCALABILITY ROADMAP-SCALABILITY ROADMAP

2024 | 2026 | 2028 | 2030 | 2032 | 2034

**2025**
Commencement of the venture in the given 3 districts

**2027**
Expansion to 30 districts; train 200+ saathis

**2029**
Collaborate with the governments on health access programs

**2031**
Expand services: diagnostics, chronic disease areas

**2033**
Enter 5+ states; establish 100+ swasthya vans